

Patient Characteristics	MDH Recommendations	Added Considerations	Patient Considerations
Extrapulmonary Only Normal CXR	No Respiratory Isolation or Restrictions	Ensure evaluation for TB of respiratory tract with chest imaging and sputum bacteriologic testing	Evaluate weekly 1.Assess Financial impact and support as resources allow 2.Assess Housing 3.Assess Mental Health and refer for additional counseling/support 4.Assess Food security
Children <10 with intrathoracic TB	No isolation except for older children and adolescents with adult-type disease	Individuals with sputum bacteriologic tests that are positive may be considered as having adult-type disease	
Low pre-treatment infectiousness (e.g., sputum smear-negative & non-cavitary) + GXP available (Rifampin S)	All settings and contacts: RIR through at least 5 days of verified treatment*	Request GXP. See below if not available.	
Moderate or High pre-treatment infectiousness (e.g., sputum smear-positive OR cavitation or extensive/multilobar) + GXP available (Rifampin S)	Lower risk settings and contacts RIR through 5-10 days of verified treatment* Higher risk settings and contacts^b: RIR through 10-14 days of verified treatment, and documented clinical response (symptom improvement) and/or microbiologic response (reducing sputum smear grade)*	1.Request GXP. See below if not available. 2.If High pre-treatment infectiousness (sm+ and cavitation) with high risk setting (e.g., vulnerable population), request MDDR to verify INH S; Consider HPMZ or high dose rifamycin to improve EBA of first line therapy	Tailor restrictions: 1.Consider Moderate restrictions in most instances (allow outdoor activities that do not involve close, prolonged contact) 2.Evaluate employment setting and make tailored recommendation)
GXP unavailable	Low bacterial burden and Lower Risk Settings: 10-14 days of verified treatment and clinical improvement* High bacterial burden OR Higher Risk Settings^b: At least 14 days of verified treatment* and clinical improvement and microbiologic response (reducing smear grade)	1.Request GXP and/or MDDR, particularly for high bacterial burden or higher risk settings 2.Collect weekly sputum x 3 to evaluate microbiologic response to assess appropriateness of treatment	
Rifampin Resistant	Minimum 14 days of laboratory confirmed effective therapy + clinical improvement, and demonstrated microbiologic response (reduced smear grade or increasing time to culture positivity on serial testing)	1.Request MDDR and phenotypic DST 2.Effective treatment is defined based on microbiological testing. Emerging data suggests BPAL/M reduces infectiousness rapidly, but data is limited. 3.For higher risk settings and contacts, a higher degree of certainty of treatment effectiveness (DST, 14-28 days of therapy, micro/clinical response) may be considered	Higher risk for negative patient impact. Evaluate as above, and engage with MDH and local social work or patient advocacy services to support patients.

^aThese guidelines do NOT apply to healthcare facilities, shelters, detention-or-correctional facilities, and analogous settings.

* Treatment includes HPMZ and HRZE for confirmed or presumed drug-sensitive TB. If alternative treatment is utilized, please consult MDH for additional guidance. Treatment includes assessment of tolerability and adherence.

Settings and Contact Considerations

^b**Patient factors** that may lead to prolonged infectiousness to consider include severe immunosuppression, poor absorption of medication (e.g., poorly controlled diabetes mellitus, vomiting, diarrhea) and uncertainty regarding adequacy of regimen. **Contact factors** warranting consideration for extended (tailored) restrictions include likely prolonged close contact children <5 or immunosuppressed individuals who have not been placed on window prophylaxis. **Community factors** include indoor spaces with poor ventilation, expected duration, frequency, and proximity of contact events with individuals not previously exposed or not included in contact investigations.